

Case Manager Scope of Work

Key Responsibilities	Daily Tasks & Workflows	Pain Points & Frustrations	Key Quotes (Cite Source)
Continuity & Transition Management	* Patient discharged from inpatient care * Discharge initiated - case manager receives (often incomplete) discharge information * Attempt to contact patient/caregiver (may require multiple attempts) within 2 days * Confirm outpatient appointment scheduling * Monitor first clinic visit attendance * Track missed appointments and follow up	* Information: Discharge details incomplete or delayed * Operational: Patients unreachable post-discharge * Authority: Cannot enforce appointment attendance * System: No single owner of transition outcomes	* <i>Paraphrase: "Discharge happens fast, but the information doesn't always come with it."</i> * <i>Paraphrase: "If they miss that first appointment, everything starts to unravel."</i>
Risk Surveillance & Early Intervention	* Identify patient under active monitoring * Conduct periodic mental state/risk assessments * Monitor compliance with medication and care plan * Detect early warning signs of deterioration * Coordinate pre-crisis intervention (safety planning) * Escalate concerns when thresholds are met	* Operational: Limited time for continuous monitoring * Information: Subtle behavioral changes are hard to detect remotely * Authority: Cannot adjust treatment directly * Emotional Load: Responsibility for catching risk early	* <i>Paraphrase: "You're trying to catch something before it becomes visible."</i> * <i>Paraphrase: "By the time it's obvious, it's already a crisis."</i>
Information Integration Across Teams	* Receive information from inpatient team * Document case updates in clinical system * Communicate care plan to outpatient providers * Liaise with caregivers and community partners * Provide psychoeducation to patient/family * Update all parties as condition evolves	* Information: Systems not interoperable * Operational: Repeating same information across stakeholders * Authority: No control over data quality from other teams * System: Reliance on manual communication, no single source of truth across care settings	* <i>Paraphrase: "Half the job is just making sure everyone is on the same page."</i> * <i>Paraphrase: "We're the middle between systems that don't talk to each other."</i>
Crisis Coordination & Escalation	* Identify crisis signal (missed contact, behavioral change) * Contact patient/caregiver to assess severity * Attempt de-escalation (guidance, reassurance) * Activate crisis services if threshold met * Coordinate with emergency response teams * Document incident and follow-up actions	* Operational: Delays in crisis response systems * Authority: Cannot directly deploy crisis resources * System: Escalation pathways unclear or inconsistent * Emotional Load: High-stakes decision making	* <i>Paraphrase: "You don't control the response, but you're responsible for triggering it."</i> * <i>Paraphrase: "Everything depends on how fast the system reacts...and we don't control that speed."</i>
Therapeutic Engagement & Family Support	* Establish rapport with patient and caregiver * Provide counseling and emotional support * Conduct home visits when necessary * Encourage treatment adherence * Send appointment reminders * Maintain ongoing communication	* Operational: High caseload limits depth of engagement * System: Trust-building is time-intensive and not scalable * Emotional Load: Managing family dynamics and distress * Outcome Dependency: Engagement directly affects compliance	* <i>Paraphrase: "If they don't trust you, they won't pick up the phone."</i> * <i>Paraphrase: "Everything works better when the relationship is strong."</i>
Documentation & Care Plan Updates	* Record all interactions in clinical system * Update care plans based on patient status * Log risk assessments and interventions * Ensure documentation meets compliance standards * Share updates with MDT * Maintain audit trail of decisions/actions	* Operational: Documentation is time-consuming * System: EHR systems are inefficient or fragmented * Conflict: Time spent documenting vs patient care * Information: Duplication of entries across systems	* <i>Paraphrase: "If it's not documented, it didn't happen."</i> * <i>Paraphrase: "You spend hours charting instead of helping."</i>